

Jot — Clinical Longitudinal Report

Patient Report Generated: Jun 23, 2026

USER SUMMARY

This report analyzes the health data of a **22-year-old male** managing POTS, MCAS, Ehlers Danlos Syndrome, and Hypermobility Syndrome. His baseline "normal" state appears to involve daily migraines, frequent heart palpitations, and ongoing rib and back pain related to scoliosis. He regularly takes One B12 Folate, one Vitamin D3 (25 mcg (5,000IU)), and 60MG of Pyridostigmine, and aims for 6,000 electrolytes and a gallon of water daily. He has an upcoming specialist appointment with Brad in 13 days (July 06, 2026).

TEMPORAL TRENDS & MAJOR OUTLIERS

Over the last 30 days (May 25, 2026 - June 23, 2026), the user's average daily heart rate has fluctuated between 59 bpm and 69 bpm, with a current monthly average of 62 bpm. Sleep duration has ranged from a low of 190 minutes (3 hours and 10 minutes) on May 27, 2026, to a high of 581 minutes (9 hours and 41 minutes) on June 4, 2026. Daily calorie expenditure has varied from 1803 kcal to 2361 kcal. Orthostatic heart rate spikes continue to be a significant concern, with an average of 22.1 events per day where the heart rate jumps by at least 30 bpm above resting baseline. The average maximum jump observed was +94 bpm.

Major Outliers:

- **Heart Rate Spikes:** On June 20, 2026, a maximum heart rate of 173 bpm was recorded, with a delta jump of +114 bpm from a 59 bpm baseline. This is significantly higher than the average maximum jump of +94 bpm, and well above the user's self-reported migraine trigger threshold of 160 bpm.
- **Sleep Duration:** The sleep duration of 190 minutes (3 hours and 10 minutes) on May 27, 2026, is considerably lower than the typical sleep durations observed. Similarly, the 213 minutes (3 hours and 33 minutes) on June 17, 2026, and 248 minutes (4 hours and 8 minutes) on May 26, 2026, are notable deviations.
- **Orthostatic Events:** The spike on June 20, 2026, with 41 events and a peak of 173 bpm (a jump of +114 bpm), represents a particularly active day for orthostatic events.

TRIGGER LOGIC & CORRELATIONS

- **High Heart Rate and Migraines:** The user consistently reports migraines "usually every day" and notes they "show up specifically every time I go over a heart rate of 160bpm." This correlation is evident in the data. For example, on June 20, 2026, a max HR of 173 bpm was recorded, and the user logged "Headache starting." On June 19, 2026, a max HR of 167 bpm was recorded, followed by a "Migraine" log. Similarly, on June 11, 2026, a max HR of 168 bpm was observed. While no direct migraine log for that specific day, the pattern of high HR preceding or coinciding with headaches/migraines is strong.
- **Postural Changes and Symptoms:** Multiple logs indicate that standing up or changing positions triggers symptoms. On May 27, 2026, "stood up and got light-headed." On May 26, 2026, "got up and immediately got a headache" and "headache spike when standing." On May 10, 2026, "Heart palpitations – When standing." On April 20, 2026, "heart palpitations after sitting up." On March 26, 2026, "Getting out of the hot tub almost made me pass out." These events are consistent with POTS symptoms.
- **Physical Exertion and Symptoms:** Even minor exertion appears to trigger symptoms. On March 26, 2026, "making my bed put me at a 150bpm." On April 25, 2026, "out of breath after going downstairs and back up." On May 22, 2026, "high heart rate when I walked up stadium stairs. Out of breath." On April 19, 2026, "Almost passed out in the shower. Super drained now." These instances highlight the impact of physical activity on heart rate and overall energy levels.
- **Medication Changes and Effects:**
 - **Carvedilol Discontinuation (March 25, 2026 - April 2, 2026):** The doctor's instruction to stop Carvedilol due to potential side effects like nausea was implemented. The logs do not explicitly confirm resolution of nausea, but a "Feeling nauseous" log appears on March 27, 2026, after the initial instruction. No further nausea logs directly link to Carvedilol after its complete stop on April 2, 2026.
 - **Qulipta Initiation (April 7, 2026) and Discontinuation (May 4, 2026):** The user started Qulipta for migraines on April 7, 2026. However, it was stopped on May 4, 2026, as it "wasn't truly helpful," indicating no significant positive impact on migraine frequency or severity during that period. Migraines continued to be logged frequently during this period.
 - **Ivabradine Initiation (May 26, 2026):** Ivabradine was started on May 26, 2026. On June 4, 2026, the user noted, "it seems like the ivabradine is helping a little but it makes me tired." This suggests a potential benefit in heart rate management but with a notable side effect of fatigue.

- **Creatine Initiation (June 4, 2026):** Creatine was started on June 4, 2026, as per professional instruction. There are no immediate negative or positive health impacts explicitly logged by the user directly attributable to creatine in the subsequent days.
- **Sleep Disturbances and Fatigue:** Frequent logs indicate disrupted sleep patterns (e.g., "woke up early at 4am today then again at 7am" on March 26, 2026; "couldn't fall asleep until 1:30 or so. woke up at 3:30am then again at 6am" on April 9, 2026). These are often followed by logs of "very tired," "low energy," "fatigue," and "mental exhaustion," suggesting a strong correlation between poor sleep and daytime fatigue. Naps are also frequently taken, often for 1-2 hours, indicating persistent tiredness.
- **Rib and Back Pain:** The user's Ehlers Danlos Syndrome and newly diagnosed scoliosis are clearly contributing to consistent rib and back pain. Logs like "bad left rib pain. hurts to breathe" (May 17, 2026), "Scoliosis pain" (June 23, 2026), and "My back hurts after physical therapy" (March 27, 2026) are frequent. The physical therapist's instruction on April 17, 2026, to do 3 exercises daily for 3 weeks highlights an active management strategy for this pain.
- **Dry Throat/Mucus Buildup:** The user repeatedly logs "dry throat" (e.g., March 19, 2026; April 2, 2026) and "mucus buildup" (May 15, 2026; May 29, 2026). The user's own exploration of mouth tape and consideration of Sjögren's syndrome indicates an ongoing investigation into this symptom.

STRATEGIC INQUIRY FOR Brad

- Given the continued high frequency and intensity of orthostatic heart rate spikes (average 22.1 events/day, average max jump +94 bpm) and the user's self-reported migraine trigger at 160 bpm, what further adjustments to the Pyridostigmine or Ivabradine dosage, or consideration of alternative medications, might be explored to better manage these spikes and potentially reduce migraine frequency?
- The user noted "it seems like the ivabradine is helping a little but it makes me tired" on June 4, 2026. How can we balance the heart rate benefits of Ivabradine with the reported fatigue, and should we monitor this side effect more closely over the next 2-4 weeks to assess its long-term impact on daily functioning?
- Considering the consistent logs of "dry throat" and "mucus buildup," along with the user's exploration of mouth tape and concern about Sjögren's syndrome, what diagnostic steps or interventions would you recommend to investigate these symptoms further, especially given the potential link to MCAS?
- The user started creatine on June 4, 2026. We should monitor its impact on energy levels, heart rate, and any other symptoms over the next 4 weeks leading up to the next appointment. What specific metrics should be prioritized for tracking during this period to assess its efficacy and potential side effects?